

SURGICAL PATHOLOGY REPORT
 UID:4BE36D8A-9841-4CC7-9331-A2A14D5C2C30
 TCGA-XF-AAN4-01A-PR

Redacted


 Collected: [REDACTED]
 Ordered by: [REDACTED]

Location: [REDACTED]

DIAGNOSIS:

RADICAL CYSTECTOMY (ANTERIOR EXENTERATION) WITH ILEAL CONDUIT:

RIGHT DISTAL URETER (A):

INTRAOPERATIVE FROZEN SECTION DIAGNOSIS:

- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

LEFT DISTAL URETER (B):

INTRAOPERATIVE FROZEN SECTION DIAGNOSIS:

- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

OVARIES, BLADDER, CERVIX, PERIVESICAL LYMPH NODES (C):

BLADDER:

- INVASIVE POORLY DIFFERENTIATED UROTHELIAL CARCINOMA WITH SIGNIFICANT GLANDULAR COMPONENT (6 X 4 X 2.5 CM), GRADES 3 AND 4/4, GROSSLY AND MICROSCOPICALLY EXTENDING THROUGH ENTIRE BLADDER WALL INTO SUBSEROSAL AND PERIVESICAL SOFT TISSUE
- UROTHELIAL CARCINOMA IN SITU (FLAT LESION) ASSOCIATED WITH INVASIVE CARCINOMA AND INVOLVING LEFT URETEROVESICAL JUNCTION
- ADDITIONAL RANDOM SECTIONS OF BLADDER INCLUDING URETHRA AND TRIGONE/BLADDER NECK, FREE OF UROTHELIAL DYSPLASIA AND CARCINOMA
- NO LYMPHOVASCULAR SPACE INVASION IDENTIFIED
- RESECTION MARGINS, FREE OF UROTHELIAL DYSPLASIA AND MALIGNANCY

UTERUS WITH CERVIX:

- CERVIX, BENIGN SQUAMOUS AND ENDOCERVICAL MUCOSA
- ENDOMETRIUM, ATROPHIC
- MYOMETRIUM, NO SIGNIFICANT HISTOPATHOLOGIC CHANGE
- SEROSA, NO SIGNIFICANT HISTOPATHOLOGIC CHANGE

BILATERAL FALLOPIAN TUBES AND OVARIES:

- LEFT OVARY, SIMPLE SEROUS CYST AND BENIGN PHYSIOLOGIC CHANGES
- RIGHT OVARY, BENIGN PHYSIOLOGIC CHANGES
- RIGHT AND LEFT FALLOPIAN TUBES, NO SIGNIFICANT HISTOPATHOLOGIC CHANGE
- NO ATYPIA OR MALIGNANCY IDENTIFIED

RIGHT PARACAVAL LYMPH NODES (D):

- NO MALIGNANCY IDENTIFIED IN SEVEN LYMPH NODES EXAMINED (0/7)

RIGHT COMMON ILIAC LYMPH NODES (E):

- NO MALIGNANCY IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

PRESACRAL LYMPH NODES (F):

- NO MALIGNANCY IDENTIFIED IN FOUR LYMPH NODES EXAMINED (0/4)

RIGHT LYMPH NODE OF CLOQUET (G):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1)

RIGHT EXTERNAL ILIAC LYMPH NODES (H):

- NO MALIGNANCY IDENTIFIED IN SEVEN LYMPH NODES EXAMINED (0/7)

RIGHT OBTURATOR / HYPOGASTRIC LYMPH NODES (I):

ICD 6.3
 Carcinoma, urothelial NOS
 812013
 Site: Bladder NOS
 C67.9
 JWS 2/26/14

Pw TSS, glandular = 5%.

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- NO MALIGNANCY IDENTIFIED IN EIGHT LYMPH NODES EXAMINED (0/8)

LEFT PARA AORTIC LYMPH NODES (J):

- NO MALIGNANCY IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

LEFT COMMON ILIAC LYMPH NODES (K):

- NO MALIGNANCY IDENTIFIED IN TWO LYMPH NODES EXAMINED (0/2)

LEFT LYMPH NODE OF CLOQUET (L):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1)

LEFT EXTERNAL ILIAC LYMPH NODES (M):

- NO MALIGNANCY IDENTIFIED IN NINE LYMPH NODES EXAMINED (0/9)

LEFT OBTURATOR / HYPOGASTRIC LYMPH NODES (N):

- NO MALIGNANCY IDENTIFIED IN 12 LYMPH NODES EXAMINED (0/12)

LEFT PRESACRAL LYMPH NODES (O):

- NO MALIGNANCY IDENTIFIED IN TWO LYMPH NODES EXAMINED (0/2)

RIGHT PRESACRAL LYMPH NODES (P):

- NO MALIGNANCY IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

LEFT PROXIMAL URETER (Q):

- BENIGN URETER

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

RIGHT PROXIMAL URETER (R):

- BENIGN URETER

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

LYMPH NODE SUMMARY: NO MALIGNANCY IDENTIFIED IN TOTAL OF 65 LYMPH
NODES EXAMINED (0/65)

PATHOLOGIC TNM STAGE: pT1sT3bN0MX

Electronically signed by
Verified:

COMMENT:

Representative sections of invasive carcinoma are submitted for p53 assay by immunohistology, results of which will be issued in an addendum report.

SPECIMEN SOURCE:

- A: "Rt distal ureter F/S"
- B: "Lt distal ureter F/S"
- C: "Ovaries, bladder, cervix, per-vessicle L.N."
- D: "Right para caval LN"
- E: "Rt common iliac LN"
- F: "Pre sacral LN"
- G: "Rt LN of cloquet"
- H: "Rt external iliac LN"
- I: "Rt obturator hypogastric LN"
- J: "Lt para aortic LN"
- K: "Lt common iliac LN"
- L: "Left lymph node of cloquet"

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M: "Left external iliac lymph nodes"
 N: "Lt obturator hypogastric LN"
 D: "Lt presciatic LN"
 P: "Rt pre sciatic LN"
 Q: "Left ureter"
 R: "Right ureter"

CLINICAL INFORMATION:

Pre-op Dx: Bladder cancer

Post-op Dx: Same

GROSS EXAMINATION:

A: The specimen is received fresh from the O.R. and labeled "Rt distal ureter F/S". It consists of a portion of ureter measuring 0.2 x 0.2 x 0.1 cm with attached fat measuring 0.3 x 0.1 cm. The specimen is entirely submitted for frozen section in AFS.

B: The specimen is received fresh from the O.R. and labeled "Lt distal ureter F/S". It consists of a portion of ureter measuring 0.2 x 0.2 x 0.1 cm with attached fat measuring 0.2 x 0.1 cm. The specimen is entirely submitted for frozen section in BFS.

C: The specimen is received fresh from the O.R. and labeled "Ovaries, bladder, cervix, per-vessicle L.N.". It consists of a cystectomy specimen with total abdominal hysterectomy, bilateral salpingo oophorectomy specimen measuring overall 19.5 x 13 x 6 cm. Attached is a peritoneal segment of fat measuring 19.5 x 13 x less than 0.1 cm. It is firm and wrinkled over an area of 1.8 cm in the left posterolateral dome region. The bladder itself measures 8 x 6 x 3 cm. Selected areas on the wrinkled peritoneum and the external surface of the dome and left posterolateral wall area are inked black. Opening the bladder along the anterior wall reveals a 6 x 4 cm gray-white tumor mass which occupies the posterior dome and the left posterolateral wall, that comes within 3 mm of the left ureterovesical junction and 4 cm from urethral margin. The tumor grossly goes through the wall and involves the perivesical soft tissue and subserosa beneath the peritoneum. It measures 2.5 cm in thickness. The involved bladder wall measures 1.5 cm in thickness and the uninvolved bladder wall measures 0.7 cm in thickness. The urethra measures 1.5 cm in length x 0.8 cm in circumference and has grossly normal appearing mucosa. The attached right ureter is probe patent, measures 4 cm in length x 0.4 cm in circumference and appears grossly unremarkable. The attached left ureter is probe patent, measures 3.5 cm in length x 0.5 cm in circumference and appears grossly unremarkable. The contiguous uterus with cervix measures 5.7 x 4.7 cm and 1.5 cm in thickness with a smooth serosa. The exocervix measures 3 cm across and the external os is slit-like and measures 1 cm. The cervical mucosa is unremarkable. The right adnexa consists of right ovary measuring 2 x 1.8 x 0.9 cm with attached fallopian tube which measures 6 cm in length x 0.3 cm in diameter. The left adnexa consists of left ovary measuring 2.6 x 1.4 x 0.8 cm and left fallopian tube measuring 7 cm in length with a diameter ranging from 0.3 up to 0.5 cm. The right and left ovaries appear grossly unremarkable with slightly grooved surfaces. The uterus is bivalved revealing an endometrial lining 0.2 cm in thickness. The myometrium measures 0.9 cm and is grossly unremarkable. Representative sections are submitted in 23 cassettes.

D: The specimen is received in formalin and labeled "Right para caval LN". It consists of fat measuring 4 x 3 x 2 cm. Sectioning reveals multiple lymph nodes. The specimen is entirely submitted in three cassettes.

E: The specimen is received in formalin and labeled "Rt common iliac LN". It consists of lymph nodes measuring 0.5 x 0.3 x 0.2 cm. The specimen is entirely embedded in one cassette.

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F: The specimen is received in formalin and labeled "Pre sacral LN". It consists of fat and fibrous tissue measuring 2 x 1 x 0.5 cm. The specimen is entirely submitted in three cassettes.

G: The specimen is received in formalin and labeled "Rt LN of cloquet". It consists of a lymph node measuring 0.5 x 0.4 x 0.4 cm. The specimen is bisected and totally embedded in one cassette.

H: The specimen is received in formalin and labeled "Rt external iliac LN". It consists of fat measuring 3 x 2 x 1 cm. The specimen is entirely submitted in three cassettes.

I: The specimen is received in formalin and labeled "Rt obturator hypogastric LN". It consists of fat measuring 2 x 1 x 0.5 cm. The specimen is entirely submitted in four cassettes.

J: The specimen is received in formalin and labeled "Lt para aortic LN". It consists of lymph nodes with attached fat measuring 1.5 x 1.5 x 1 cm. The specimen is entirely submitted in three cassettes.

K: The specimen is received in formalin and labeled "Lt common iliac LN". It consists of fat measuring 1 x 0.5 x 0.5 cm. Sectioning reveals one lymph node. The specimen is entirely submitted in two cassettes.

L: The specimen is received in formalin and labeled "Left lymph node of cloquet". It consists of lymph nodes with attached fat measuring 1 x 0.7 x 0.5 cm. The specimen is entirely submitted in two cassettes.

M: The specimen is received in formalin and labeled "Left external iliac lymph nodes". It consists of multiple fragments of fat measuring in aggregate 2 x 1 x 0.5 cm. The specimen is entirely submitted in three cassettes.

N: The specimen is received in formalin and labeled "Lt obturator hypogastric LN". It consists of fat measuring 3 x 2 x 1 cm. The specimen is entirely submitted in four cassettes.

O: The specimen is received in formalin and labeled "Lt presciatic LN". It consists of multiple fragments of fat measuring in aggregate 1 x 0.5 x 0.2 cm. The specimen is entirely submitted in three cassettes.

P: The specimen is received in formalin and labeled "Rt pre sciatic LN". It consists of fat measuring 2 x 1 x 0.5 cm. The specimen is entirely submitted in three cassettes.

Q: The specimen is received in formalin and labeled "Left ureter". It consists of a portion of ureter measuring 0.7 x 0.4 x 0.3 cm with attached fibrous tissue. The specimen is totally embedded in one cassette.

R: The specimen is received in formalin and labeled "Right ureter". It consists of a portion of ureter measuring 0.3 x 0.2 x 0.2 cm with staples. The specimen is entirely embedded in one cassette.

SECTIONS:

- AFS: frozen section, right distal ureter
- BFS: frozen section, left distal ureter
- C1: ovaries, bladder, cervix, perivesical lymph nodes; urethral margin
- C2: anterior bladder wall
- C3: right lateral bladder wall
- C4: right ureterovesical junction
- C5: tumor with serosa

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C6-7: one continuous section of tumor from tumor surface to inked surgical margin
 C8: tumor with mucosa
 C9: tumor with inked surgical margin
 C10: tumor with inked surgical margin
 C11: tumor with mucosa
 C12: posterior bladder wall
 C13: left ureterovesical junction
 C14: trigone/bladder neck
 C15: urethra
 C16: right fallopian tube and ovary
 C17: left fallopian tube and ovary
 C18: anterior cervix
 C19: posterior cervix
 C20: anterior endomyometrium
 C21: posterior endomyometrium
 C22: right perivesical fat for possible lymph nodes
 C23: left perivesical fat for possible lymph nodes
 D1: right para caval lymph nodes - all embedded
 D2-3: one lymph node - all embedded
 E: right common iliac lymph nodes - all embedded
 F1-3: presacral lymph nodes - all embedded
 G: right lymph node of Cloquet, bisected - all embedded
 H1: right external iliac lymph nodes, one lymph node
 H2-3: remaining tissue - all embedded
 I1-4: right obturator hypogastric lymph nodes - all embedded
 J1-2: left para aortic lymph nodes, one lymph node each
 J3: remaining tissue - all embedded
 K1-2: left common iliac lymph nodes, one lymph node each - all embedded
 L1: left lymph node of Cloquet
 L2: remaining tissue - all embedded
 M1-3: left external iliac lymph nodes - all embedded
 N1-4: left obturator hypogastric lymph nodes - all embedded
 O1-3: left presciatic lymph nodes - all embedded
 P1-3: right presciatic lymph nodes - all embedded
 Q: left proximal ureter - all embedded
 R: right proximal ureter - all embedded

INTRAOPERATIVE FROZEN CONSULTATION:

AFS: Right distal ureter
 - no high grade atypia or tumor identified
 BFS: Left distal ureter
 - no high grade atypia or tumor identified

MICROSCOPIC EXAMINATION:

A-B: See final microscopic-diagnosis.

C: Sections of the bladder show an invasive poorly differentiated urothelial carcinoma with significant glandular features (C5-12), grades 3 and 4/4. The tumor grossly and microscopically extends through the entire bladder wall to involve subserosal and perivesical soft tissue. Urothelial carcinoma in situ (flat lesion) is identified adjacent to the main tumor mass (C8,12) and more remotely in the left ureterovesical junction (C13). All resection margins are free of both urothelial

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dysplasia and malignancy and no lymphovascular space invasion is identified. Additional random sections of the bladder that include the urethra and trigone/bladder neck show no evidence of urothelial dysplasia or carcinoma.

D-R: See final microscopic-diagnosis.

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case Is (circle):	QUALIFIED / DISQUALIFIED	
Reviewer Initials	Date Reviewed: 1/16/2014	